# Obesity Medications — Detailed Reference

Dosing schedules, mechanisms of action, contraindications, and drug interactions for all anti-obesity medications

## LIRAGLUTIDE (GLP-1 AGONIST) — RX · SC DAILY

### Liraglutide

*Saxenda® (obesity) · Victoza® (diabetes)*

**Titration:** Wk1: 0.6mg → Wk2: 1.2mg → Wk3: 1.8mg → Wk4: 2.4mg → Wk5+: 3.0mg/day SC
**Inject:** Abdomen, thigh, or upper arm — site & timing changeable
**Action:** GLP-1 receptor agonist
**SE:** Nausea, diarrhea, vomiting, headache, dizziness, fatigue, renal insufficiency
**CI:** **Medullary thyroid CA (personal/family), MEN2, pancreatitis, gallbladder disease, suicidal ideation, renal failure, pregnancy**
**Interact:** Slows gastric emptying — affects oral med absorption; serious hypoglycemia with sulfonylureas
**Age:** ≥12 for obesity

## METFORMIN — RX · PO DAILY

### Metformin

*Glucophage® · off-label weight use*

**Dose:** Start 500mg PO QD → titrate to 2000mg QD
**Action:** Exact weight loss mechanism unknown; improves insulin sensitivity
**SE:** Lactic acidosis (severe — onset may be subtle: malaise, myalgias, somnolence, abdominal pain); hypothermia, hypotension, bradyarrhythmias in overdose
**CI:** **eGFR <30, metabolic acidosis, DKA, lactic acidosis, hypoxemia, dehydration. Use caution if eGFR 30–45**
**Interact:** MATE1/MATE2-K/OCT1/OCT2 substrate; antidiabetic agents; hold before radiocontrast/surgery
**Age:** ≥10 for T2DM

## NALTREXONE/BUPROPION — RX · PO

### Naltrexone / Bupropion ER

*Contrave® · 8mg/90mg tablets*

**Titration:** Wk1: 1 tab AM · Wk2: 1 AM + 1 PM · Wk3: 2 AM + 1 PM · Wk4+: 2 AM + 2 PM
**Action:** Naltrexone (opioid antagonist) + bupropion (antidepressant/cessation aid)
**SE:** Nausea, constipation, headache, vomiting, diarrhea, acute closure glaucoma, dizziness, insomnia, dry mouth
**CI:** **Acute angle-closure glaucoma, uncontrolled HTN, seizure disorder, opioid use, drug/alcohol withdrawal, pregnancy; antidepressants may ↑ suicidal thinking in young adults**
**Interact:** Cannot combine with opioids; MAO inhibitors; seizure-lowering drugs; Digoxin ↓; monitor glucose with antidiabetics
**Age:** ≥18 for obesity

## ORLISTAT — OTC: ALLI 60MG · RX: XENICAL 120MG

### Orlistat

*Xenical® 120mg (Rx) · Alli® 60mg (OTC)*

**Dose:** Xenical: 120mg TID with fatty meals (during or ≤1hr after). Alli: 60mg with fatty meals
**Action:** GI lipase inhibitor — impairs dietary fat digestion/absorption
**SE:** Oily anal discharge/flatus after fatty foods, gallstones, kidney stones, fat-soluble vitamin malabsorption (A, D, E, K)
**CI:** **Chronic malabsorption syndrome, cholestasis, warfarin use, hypersensitivity**
**Interact:** Cyclosporine, hormonal contraceptives, seizure meds, thyroid hormones, warfarin
**Age:** ≥12 for obesity

## PHENTERMINE — RX · DEA SCHED IV · PO

### Phentermine

*Adipex-P® · Lomaira® · Suprenza® · Approved short-term (12 wks)*

**Dose:** 15–37.5mg PO qAM (before or 1–2hr after breakfast); max 37.5mg/day; taper to D/C if prolonged use
**Action:** Sympathomimetic amine — appetite suppressant (norepinephrine release)
**SE:** Headache, ↑BP, rapid/irregular HR, overstimulation, tremor, insomnia
**CI:** **Hyperthyroidism, hypertension, renal failure, seizure disorder, CAD, glaucoma, agitation, drug abuse, pregnancy, MAOI within 14 days**
**Interact:** MAOIs (dangerous), sympathomimetics, alcohol, anesthetics; may reduce insulin/OHA dosing needs; ↓ effect of adrenergic blockers
**Age:** ≥16 for obesity

## PHENTERMINE/TOPIRAMATE ER — RX · DEA SCHED IV · QAM

### Phentermine / Topiramate ER

*Qsymia® · ⚠ Teratogenic — monthly pregnancy test required*

**Titration:** Start: 3.75/23mg × 14 days → Target: 7.5/46mg → Titrate: 11.25/69mg → Max: 15/92mg QAM · Taper to D/C — never abrupt stop (seizure risk)
**Action:** Phentermine (sympathomimetic) + topiramate (anticonvulsant/migraine)
**SE:** Paresthesia (tingling), dizziness, dysgeusia, insomnia, constipation, dry mouth, ↑HR, suicidal ideation, mood/cognitive/sleep disorders, metabolic acidosis, elevated creatinine, kidney stones, hypokalemia, oligohidrosis/hyperthermia
**CI:** **Pregnancy (orofacial cleft teratogen!), acute myopia/glaucoma, hyperthyroidism, MAOI ≤14 days, hypersensitivity**
**Interact:** MAOIs; alcohol (potentiates CNS depression); non-K-sparing diuretics (hypokalemia); OCP (irregular bleeding, not ↑pregnancy risk); monitor electrolytes
**Age:** ≥18 for obesity

## SEMAGLUTIDE (GLP-1 AGONIST) — RX · SC WEEKLY

### Semaglutide

*Wegovy® (obesity) · Ozempic® (T2DM)*

**Wegovy:** 0.25→0.5→1→1.7→2.4mg SC qwk (4-wk intervals each step) · ~15% avg weight loss
**Ozempic:** 0.25→0.5→1→2mg SC qwk (diabetes dosing)
**Action:** GLP-1 receptor agonist
**SE:** Nausea, diarrhea, vomiting, constipation, abdominal pain, headache, fatigue, dyspepsia, belching, flatulence, GERD
**CI:** **Medullary thyroid CA (personal/family), MEN2, pancreatitis, gallbladder disease, diabetic retinopathy, pregnancy**
**Interact:** Hypoglycemia risk with sulfonylureas or insulin
**Age:** ≥18 for obesity

## TIRZEPATIDE (GLP-1 + GIP DUAL AGONIST) — RX · SC WEEKLY

### Tirzepatide ⭐ Highest weight loss

*Zepbound® (obesity) · Mounjaro® (T2DM) · ~20–25% avg weight loss*

**Titration:** 2.5→5→7.5→10→12.5→15mg SC qwk (4-wk intervals each step) · Titrate to highest tolerated dose
**Action:** Dual GLP-1 + GIP receptor agonist — unique dual mechanism
**SE:** Nausea, diarrhea, vomiting, constipation, abdominal pain, headache, fatigue
**CI:** **Medullary thyroid CA (personal/family), MEN2, pancreatitis, gallbladder disease, diabetic retinopathy, pregnancy**
**Interact:** Hypoglycemia risk with sulfonylureas or insulin
**Age:** ≥18 for obesity

## Medication Is One Piece of the Puzzle

All anti-obesity medications work best as part of a comprehensive program including reduced-calorie diet, physical activity, and behavioral counseling. Dosing is individualized — your provider will select the optimal medication and titration schedule for your specific health needs. Questions? (281) 557-0300